

Ostrander Regional Medical Center (employee plan) Employee Health Benefit Plan

Summary Plan Description (SPD) and Plan Document | Plan year 2026 | Document ID SPD-OSTR

1. Introduction

Ostrander Regional Medical Center (employee plan) (the 'Plan Sponsor') sponsors this self-funded group health plan for about 5,200 employees and 12,600 covered lives. The Plan is governed by the Employee Retirement Income Security Act of 1974 (ERISA). Because the Plan is self-funded, benefits are paid from the Plan Sponsor's assets, not by an insurance company, and state insurance laws generally do not apply to the Plan. Bellcourt Health Administrators, Inc. is the third-party administrator (TPA) and performs claims administration and utilization management, including prior authorization, on the Plan Sponsor's behalf. Bellcourt does not insure the Plan.

2. Eligibility

Full-time employees working 30 or more hours per week are eligible on the first of the month after 30 days of employment. Spouses and dependent children to age 26 may be enrolled. Coverage ends on the last day of the month in which employment ends.

3. Network

Exclusive provider organization (EPO); in-network only except emergencies.

4. Schedule of benefits

4.1 Summary

Benefit	In-network	Out-of-network
Deductible (individual / family)	\$1,500 / \$3,000	Not covered
Coinsurance	20%	Not covered
Out-of-pocket maximum	\$5,000 / \$10,000	Not covered
Advanced imaging	20% after deductible, prior authorization required	40% after deductible
Durable medical equipment	20% after deductible	40% after deductible

4.2 Outpatient rehabilitation

Outpatient physical, occupational and speech therapy are limited to a combined **30 visits per plan year**. Visits beyond the 12th require prior authorization and are reviewed under Bellcourt medical policy MP-114. Visits above the annual limit are not covered regardless of medical necessity.

5. Utilization management and prior authorization

5.1 Clinical criteria

Medical necessity is determined using the Bellcourt medical policy library in effect on the date of service, subject to any stricter or more specific provision of this Plan. Where this Plan and a Bellcourt medical policy conflict, this Plan controls.

5.2 Services requiring prior authorization

- Advanced imaging (MRI, CT angiography)
- Inpatient admissions and all elective surgery
- Durable medical equipment over \$500 (including CGM and CPAP)
- Sleep studies performed in a facility
- Genetic testing
- Hyperbaric oxygen therapy
- Radiation therapy including proton beam
- Home health services
- Outpatient physical, occupational and speech therapy beyond the 12th visit in a plan year

5.3 Centers of Excellence

Reserved.

5.4 Decision timeframes

Urgent pre-service requests: decision within 72 hours. Non-urgent pre-service requests: decision within 15 days, extendable once by 15 days where the extension is necessary and the member is notified.

5.5 Adverse determinations

Any denial based on medical necessity must be made by a licensed physician. The notice must state the specific reason, the plan provision or clinical criteria relied upon, and the member's appeal rights.

6. Exclusions and limitations

The following are not covered, or are covered only as described:

Section	Item	Provision
6.1	Cosmetic services	Services primarily intended to improve appearance, including cosmetic rhinoplasty and cosmetic eyelid surgery.
6.2	Eyelid surgery	Covered only when criteria in the applicable Bellcourt medical policy are met.
6.3	Experimental and investigational services	Services considered experimental or investigational under Bellcourt medical policy MP-117.
6.4	Bariatric surgery	Covered subject to Bellcourt medical policy MP-102.
6.5	Infertility treatment	Covered up to a \$15,000 lifetime maximum.
6.6	Custodial care	Assistance with activities of daily living without a skilled need.
6.7	Proton beam therapy	Covered subject to Bellcourt medical policy MP-113.

7. Claims and appeals

Members and providers may appeal an adverse determination within 180 days. Appeals of medical necessity decisions are reviewed by a physician not involved in the original decision. External review is available after internal appeals are exhausted.

8. Amendments

No amendments for the current plan year.